

1. Administrative & Study Identification

Full Study Title: An Open-label, Randomized, Controlled Phase 3 Study of Disitamab Vedotin in Combination With Pembrolizumab Versus Chemotherapy in Subjects With Previously Untreated Locally Advanced or Metastatic Urothelial Carcinoma That Expresses HER2 (IHC 1+ and Greater) **Short Title/Acronym:** Disitamab Vedotin With Pembrolizumab vs Chemotherapy in Previously Untreated Urothelial Cancer Expressing HER2 **Protocol Number:** SGNDV-001 / PF-08046051 **NCT Number:** NCT05911295 **Sponsor/Company Name:** Pfizer **Investigational Product:** Disitamab Vedotin + Pembrolizumab **Phase of Development:** Phase 3 **Trial Status:** Recruiting **Medical Monitor:** Pfizer Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate whether the combination of the antibody-drug conjugate (ADC) disitamab vedotin and the PD-1 inhibitor pembrolizumab provides superior progression-free survival (PFS) and overall survival (OS) compared to standard-of-care platinum-containing chemotherapy in patients with previously untreated locally advanced or metastatic urothelial cancer (la/mUC) expressing HER2. **Secondary Objectives:** To evaluate additional efficacy measures (e.g., Objective Response Rate, Duration of Response) and assess the safety and tolerability profile of taking these drugs concurrently.

2.2 Background & Rationale Locally advanced or metastatic urothelial carcinoma (la/mUC) is an aggressive disease with a significant unmet medical need and an average life expectancy of under 2 years. While platinum-based chemotherapy is the standard first-line (1L) therapy, the majority of patients progress during or after this treatment. Human epidermal growth factor receptor 2 (HER2) expression is present in approximately half of UC patients and is a biomarker associated with poor outcomes. Disitamab vedotin (an MMAE-payload ADC) combined with a PD-1 inhibitor has shown robust early-stage activity and a manageable safety profile, providing a strong rationale for this Phase 3 trial to improve first-line outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Platinum-containing chemotherapy: gemcitabine + cisplatin or carboplatin) **Blinding:** Open-label **Randomization:** 1:1 Randomization

3.2 Planned Enrollment & Duration Target **\$N\$:** 400 participants
Number of Sites: Global, multicenter (including United States, Canada, Australia, UK, and multiple countries across Europe, Asia, and South America) **Estimated Study Start:** 22-Sep-2023
Estimated Primary Completion: 30-Jun-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histopathological confirmation of locally advanced unresectable or metastatic urothelial carcinoma (la/mUC), including UC originating from the renal pelvis, ureters, bladder, or urethra. 2. Measurable disease by investigator assessment per RECIST v1.1. 3. Participant must not have received prior systemic therapy for la/mUC (exceptions made for neoadjuvant or adjuvant therapy if disease recurrence/progression occurred >12 months after the last dose). 4. Eligible to receive cisplatin- or carboplatin-containing chemotherapy. 5. Tumor expresses HER2 (IHC 1+ or greater) on immunohistochemistry. 6. Able to provide archived formalin-fixed paraffin-embedded tumor tissue blocks from a muscle-invasive or metastatic UC lesion or biopsy of metastatic UC prior to treatment initiation. If archival tissue is not available, a newly obtained baseline biopsy of an accessible tumor lesion is required within 28 days of cycle 1 day 1. 7. Eastern Cooperative Oncology Group (ECOG) performance score of 0, 1, or 2 within 7 days prior to randomization.

4.2 Exclusion Criteria 1. Known hypersensitivity to disitamab vedotin, cisplatin, carboplatin, gemcitabine, or pembrolizumab or any of their components. 2. History of or active autoimmune disease requiring systemic treatment in the past 2 years. 3. History of severe or life-threatening immune-related adverse event (irAE) with PD-(L)1 inhibitors. 4. Central nervous system (CNS) and/or leptomeningeal metastasis. Participants with treated CNS metastases are permitted if clinically stable for at least 4 weeks, baseline scans show no new/worsening metastasis, and the participant is on a stable dose of ≤ 10 mg/day of prednisone or equivalent for at least 2 weeks. 5. Prior treatment with an agent directed to another stimulatory or co-inhibitory T cell receptor (including but not limited to CD137 agonists, CAR-T cell therapy, CTLA-4 inhibitors, or OX-40 agonists). 6. Prior solid organ or bone marrow transplantation. 7. Pleural effusion or ascites with symptoms or requiring symptomatic treatment. 8. Estimated life expectancy <12 weeks. 9. Prior treatment with an MMAE agent or anti-HER2 therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Disitamab vedotin administered once every 2 weeks + Pembrolizumab administered once every 6 weeks. **Route of Administration:** Intravenous (IV)

infusion. **Duration of Treatment:** Continued until disease progression or intolerable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted (Control Arm):

Standard of care arm receives gemcitabine (once a week for 2 weeks) with either cisplatin or carboplatin (once every 3 weeks). Maintenance therapy is permitted for eligible control-arm patients where approved. **Prohibited:** Prior treatment with agents directed to other stimulatory or co-inhibitory T cell receptors, prior MMAE agents, prior anti-HER2 therapy, or previous solid organ/bone marrow transplantation.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Days -28 to 0	Informed Consent, Medical History, Tumor biopsy/archival sample, Baseline Scans (MRI/CT).
Treatment (Q2W/Q6W)	Throughout	IV infusions of Disitamab vedotin (Q2W) and Pembrolizumab (Q6W), adverse event monitoring, safety labs.
Tumor Assessment 1	Week 8	Efficacy Assessment (RECIST v1.1) via BICR.
Ongoing Assessments	Every 8 Weeks	Radiologic evaluation (every 8 weeks up to Week 72).
Long-Term Monitoring	> Week 72	Radiologic evaluation every 12 weeks.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Progression-Free Survival (PFS): The time from randomization to first documentation of disease progression per RECIST v1.1 by Blinded Independent Central Review (BICR), or to death due to any cause. **Overall Survival (OS):** The time from the date of randomization to the date of death due to any cause.

7.2 Secondary Endpoints - Objective Response Rate (ORR) per RECIST v1.1 by BICR. - Objective Response Rate (ORR) per RECIST v1.1 by investigator assessment. - Duration of Response (DOR) per RECIST v1.1 by BICR. - Safety and incidence of treatment-emergent adverse events.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular examination by medical personnel, blood tests, and cardiograms to test side effects of the drug combination. **Serious Adverse Events (SAEs):** Protocol standard reporting (within 24 hours of site awareness). **Data Monitoring Committee (DMC):** Independent central review and internal sponsor safety monitoring.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes (PFS/OS). Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target enrollment of \$N = 400\$ participants randomized 1:1 to detect statistically significant improvements in PFS and OS compared to standard chemotherapy. **Handling of Missing Data:** Standard survival analysis methodologies (e.g., Kaplan-Meier estimates) with appropriate censoring for PFS and OS.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and under the oversight of global regulatory health authorities. **Monitoring Plan:** Standard phase 3 onsite and remote monitoring for adherence to the clinical protocol and RECIST v1.1 evaluation criteria. **Audit Trail:** Protected health information protocols and standard eCRF clinical trial management systems handled by Pfizer.

11. Study Identifiers & Governance

Primary ID: SGNDV-001 **NCT Number:** NCT05911295 **Posting ID:** 706472077630243154 **Sponsor/Lead Organization:** Pfizer **Study Title:** Disitamab Vedotin With Pembrolizumab vs Chemotherapy in Previously Untreated Urothelial Cancer Expressing HER2 **Official Regulatory Title:** An Open-label, Randomized, Controlled Phase 3 Study of Disitamab Vedotin in Combination With Pembrolizumab Versus Chemotherapy in Subjects With Previously Untreated Locally Advanced or Metastatic Urothelial Carcinoma That Expresses HER2 (IHC 1+ and Greater)

12. Clinical Framework (Oncology Specific)

Disease Area: Urothelial Carcinoma **Primary Condition:** Locally Advanced or Metastatic Urothelial Carcinoma (la/mUC) **Diagnosis Threshold:** HER2 expression of IHC 1+ or greater **Medical Methodology:** HER2-directed Antibody-Drug Conjugate (ADC) + PD-1 Inhibitor vs. Platinum-based Chemotherapy **Optimization Target:** 1L (First-line) Therapy Efficacy and Survival Outcomes

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathway with interval imaging **Central Testing Protocol:** Centralized HER2 IHC testing **Assessment Protocol:** Blinded Independent Central Review

(BICR) and investigator assessment for efficacy determination
Safety Monitoring: Routine clinical labs, electrocardiograms, and symptom tracking

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression, intolerable toxicity, or study completion (Estimated completion: April 2029). **Onsite Visit Cadence:** Every 2 weeks for active treatment administration. **Tumor Assessment Frequency:** Every 8 weeks until Week 72; thereafter every 12 weeks. **Survival Follow-up:** Ongoing post-disease progression to assess Overall Survival.

15. Sign-off & Approval

	Role		Name		Signature		Date			:---		:---		:---		:---	
	Principal Investigator		[Investigator Name]		_____		[DD-MMM-YYYY]			Clinical Operations Lead		[Sponsor Representative]		_____		[DD-MMM-YYYY]	
										Lead Statistician		[Statistician Name]		_____		[DD-MMM-YYYY]	